Response to CMS–0042–NC
Request for Information: Health Technology Ecosystem
Centers for Medicare & Medicaid Services (CMS)
Assistant Secretary for Technology Policy (ASTP) / Office of the National Coordinator for Health Information Technology (ONC)
Department of Health and Human Services (HHS)

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RFI Question Mapping Table

B. Patients and Caregivers

RFI Question	Question Text (Abbreviated)	Section(s) in Response
PC-1	Health management/care navigation apps to help patients/caregivers	IV.A, V.B
PC-4	Features missing from current apps	IV.A, V.B
PC-5	How can CMS encourage patient/caregiver interest in digital health products?	IV.A, V.B
PC-8	What health data is available/valuable to patients/caregivers?	IV.C, V.B
PC-9	What additional CMS data should be included in Blue Button 2.0 API?	IV.C
PC-10	How is TEFCA helping patient access?	IV.C
PC-12	Most valuable operational health data use cases for patients/caregivers	IV.A, V.B, VI.B
PC-14	Digital identity credentials: challenges, benefits, and adoption	IV.C

C. Patients and Caregivers

RFI Question	Question Text (Abbreviated)	Section(s) in Response
PR-1	How can CMS encourage providers to leverage digital health products?	IV.A
PR-5(g)	Which FHIR APIs/capabilities do you support? (Bulk FHIR/group ID)	IV.C
PR-6(b)	Is TEFCA helping provider access? (What changes would you suggest?)	IV.C
PR-9	How might CMS encourage providers to accept digital identity credentials?	IV.C
PR-12	Should ONC revise information blocking exceptions?	VI.B

D. Payers

RFI Question	Question Text (Abbreviated)	Section(s) in Response
PA-1	Policy/technical limitations in TEFCA	IV.B, IV.C
PA-2	How can CMS encourage payers to implement APIs?	IV.B, IV.C, V.C
PA-5(a)	How can payers help simplify quality data responsibilities? (Interest in bulk extraction)	IV.C, V.C
PA-5(b)	How can quality reporting be consolidated? (Technologies for real-time quality data)	IV.C, V.C
PA-5(c)	In what ways can the interoperability and quality reporting be consolidated?	IV.B

E. Technology Vendors, Data Providers, and Networks

RFI Question	Question Text (Abbreviated)	Section(s) in Response
TD-2(a)	What additional CMS data would be valuable via APIs?	IV.C, V.B
TD-2(b)	What data sources are most valuable alongside Blue Button 2.0 API?	IV.C, V.B
TD-2(c)	What obstacles prevent accessing these data sources?	IV.C, V.B
TD-3	Digital identity implementation: challenges, benefits, impact	IV.C
TD-8	What are the most effective certification criteria and standards?	VI.A
TD-9(a)	Certification of health IT: Benefits of API-enabled capabilities	V.A, V.B, VI.A
TD-9(c)	Certification: How to require APIs to support all aspects of patient chart?	VI.A
TD-9(d)	Certification: Policy changes to motivate provider API response	VI.A
TD-15(a)	Bulk FHIR APIs: How would increased use improve use cases/data flow?	IV.C
TD-18(a)	Information blocking: Examples of practices that may constitute information blocking	II.A, II.B
TD-18(c)	Information blocking: Are there actors lacking disincentives?	II.A, II.B, VI.B

F. Value-Based Care Organizations

RFI Question	Question Text (Abbreviated)	Section(s) in Response
VB-1(a)	Incentives for APMs/ACOs to adopt digital health	IV.B, IV.D, V.A, V.B
VB-1(b)	Obstacles to broader digital product adoption for ACO patients	IV.D
VB-2	How can key technologies be better integrated into APMs?	IV.D
VB-3(c)	Essential health IT capabilities for value-based care (other capabilities)	IV.D, V.A, V.B, V.C
VB-4	Essential data types for value-based care	IV.B, IV.C, IV.D
VB-5	How do ONC certification criteria support value-based care?	VI.A, IV.D
VB-9	Technology requirements for APM vs. non-APM organizations	IV.D
VB-11	Interoperability challenges in value-based care	II.A, IV.D
VB-13	Improvements to criteria/standards for value-based care	IV.D, V.A, V.B
VB-14	Digital identity credentials for value-based care	IV.D, V.A, V.B
VB-15(a)	Nationwide provider directory of FHIR endpoints (key data elements)	IV.D, V.A, V.B

Addressing Critical DMEPOS System Inefficiencies Through Direct Physician Engagement

I. Executive Summary

Rx Redefined respectfully submits these comments regarding documented inefficiencies in the Durable Medical Equipment, Prosthetics, Orthotics, and Supplies (DMEPOS) sector, which are associated with adverse impacts on Medicare beneficiary access to timely care and increased costs to the healthcare system. The Medicare Comprehensive Error Rate Testing (CERT) program's identification of a 21.4% improper payment rate, representing $1.9 billion in projected improper payments in DMEPOS—close to triple the 7.6% rate for Part A provider services—demonstrates the need for comprehensive reform through direct physician engagement and integrated technology solutions.[1]

Analysis of current data suggests that the current third-party supplier model may perpetuate systemic inefficiencies that could persist, highlighting a potential need for structural updates to DMEPOS delivery mechanisms to better support Medicare beneficiaries and reduce instances of fraud, waste, and abuse.

II. Background: Critical Problems in Current DMEPOS Delivery

A. Patterns Consistent with Fraud, Waste, and Abuse

(Responses to VB-11, TD-18(a)(c), TD-2(d))

The DMEPOS sector demonstrates vulnerabilities that impact both the integrity of patient care delivery and fiscal management. Recent data demonstrates the severity of these challenges:

Concerning Billing Patterns: The National Association of Accountable Care Organizations (NAACOS) reported that payments for urinary catheters increased from $153 million in 2021 to $2.1 billion in 2023—representing a 1,273% increase that may account for nearly one-fifth of all Medicare spending on medical supplies in 2023. Some suppliers demonstrated billing patterns identified by regulatory authorities as possibly indicative of improper billing, with total claims approaching $3 billion, including seven high-volume suppliers charging $2 billion for catheters alone.[2,3,4]

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[1] CMS. (2024). *2024 Medicare Fee-for-Service Supplemental Improper Payment Data*. https://www.cms.gov/files/document/2024-medicare-fee-service-supplemental-improper-payment-data.pdf

[2] CMS. (2024). Mitigating the Impact of Significant, Anomalous, & Highly Suspect Billing Activity on Medicare Shared Savings Program Financial Calculations in CY 2023 Proposed Rule. https://www.cms.gov/training-education/medicare-learning-network/newsletter/2024-07-11-mlnc

[3] AHA. (2024). Coalition urges CMS to hold ACOs harmless for anomalous catheter spending. https://www.aha.org/news/headline/2024-04-30-coalition-urges-cms-hold-acos-harmless-anomalous-catheter-spending

[4] NAACOS. (2024). NAACOS Request: Hold ACOs Harmless for Anomalous Catheter Spending. https://www.hcinnovationgroup.com/policy-value-based-care/accountable-care-organizations-acos/news/55021977/naacos-request-hold-acos-harmless-for-anomalous-catheter-spending

Systemic Payment Errors: The 21.4% improper payment rate in DMEPOS, representing $1.9 billion in projected improper payments, as measured by the CERT program, greatly exceeds error rates in other Medicare sectors and may reflect underlying structural limitations requiring further attention.[5]

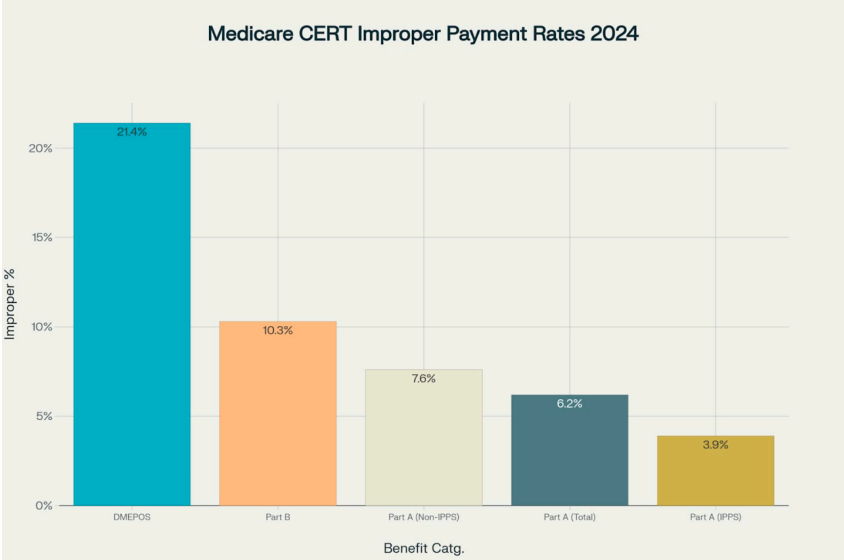

Figure 1: DMEPOS improper payment rates are nearly triple those of other Medicare benefit categories. Source: CMS. (2024). 2024 Medicare Fee-for-Service Supplemental Improper Payment Data, Table A1.

Patterns consistent with improper HCPCS code upgrading: Suppliers have at times upgrade prescriptions from lower-reimbursed codes (e.g., A4351 at $2 per catheter) to higher-reimbursed codes (e.g., A4353 at $7.50 per catheter) by soliciting medical histories to identify any record of urinary tract infections. Existing evidence does not widely support improved outcomes with higher-cost alternatives in all populations.[6]

B. Operational Inefficiencies Harming Patient Care

(Responses to VB-1(b), VB-11, PC-4(a)(c), PC-8(b)(c)(d), PC-9(b), PR-1(b), PA-1, TD-18(a)(c), TD-2(d))

Current DMEPOS workflows can create notable administrative burdens on both patients and physicians:

Excessive Administrative Loops: The typical DMEPOS order requires multiple handoffs between physicians, third-party suppliers, and payers, creating delays of up to 48 hours for prescription processing and up to seven days for product delivery. These delays particularly impact Medicare beneficiaries managing chronic conditions who require consistent access to

[5] CMS. (2024). 2024 Medicare Fee-for-Service Supplemental Improper Payment Data. https://www.cms.gov/files/document/2024-medicare-fee-service-supplemental-improper-payment-data.pdf
[6] CMS. (2021). Urological Supplies - Policy Article (A52521). https://downloads.cms.gov/medicare-coverage-database/lcd_attachments/38524_5/UrologicalSuppliesPADraft.pdf

medical supplies.[7] The primary method of communication between physicians and third-party suppliers is low tech (usually fax), which can increase coordination challenges.

Inappropriate Product Substitution Practices: Third-party suppliers sometimes substitute prescribed products with non-original alternatives, which may not always align with the prescriber's clinical intent and can impact patient familiarity or training. Requests for HCPCS code-only prescriptions and communication with patients regarding insurance coverage may, in some cases, lead to decisions that do not fully reflect the prescriber's recommendations or complete coverage details.

[7] CMS. (2023). Supplemental Report on Intermittent Urinary Catheters, p. 22. https://www.cms.gov/files/document/supplemental-report-intermittent-urinary-catheters.pdf

Provider Burden and Uncompensated Work: Physicians and clinical support staff frequently perform uncompensated administrative activities related to DMEPOS, including:

- Comprehensive patient evaluation encompassing medical history, insurance coverage criteria, and social determinants of health
- Product selection and patient training on optimal DMEPOS utilization
- Prescription formulation and transmission to third-party suppliers
- Management of documentation requests
- Ongoing patient support throughout the ordering process
- Communication bridging between patients and suppliers regarding order status

This workload often diverts time and resources from direct patient care, creating operational strain on practices serving Medicare beneficiaries with chronic conditions.

IV. Proposed Solutions

A. Direct Physician DMEPOS Provision Model

(Responses to PC-1,PC-4(a)(c), PC-5, PC-12(a)(b)(c)(d), PR-1(a)(c)(d))

Recommendation: We recommend CMS evaluate options to support and further streamline the established pathway for physicians to directly supply DMEPOS to their patients via integrated, compliant technology platforms. Such an approach has the potential to reduce reliance on third-party intermediaries and support timelier patient care.

Implementation Framework: Physicians would obtain required DMEPOS supplier enrollment and licensure by complying with the requirements in 42 CFR § 424.57 (supplier standards) and 42 CFR §§ 424.500–424.570 (enrollment process), including submission of the CMS-855S application and adherence to the CMS Publication 100-08 (Medicare Program Integrity Manual).[8,9,10,11] Certified technology platforms could be leveraged to manage billing, logistics, and compliance, ensuring adherence to applicable statutes such as Stark Law and the Anti-Kickback Statute.[12,13] This approach places clinical decision-making with the physician while supporting regulatory oversight.

B. Streamlining Payer-Provider Workflows for Value and Savings

(Responses to PA-1, PA-2, PA-5(c), VB-1(a), VB-4)

Reducing Plan Costs through Direct Physician Supply: Direct physician provision has the potential to reduce unnecessary utilization, fraud, and administrative overhead by minimizing third-party involvement. CERT data shows DMEPOS improper payments are nearly triple those of physician services, at $1.9B annually. NAACOS reports that direct physician oversight prevents increased utilization and billing not aligned with clinical needs, resulting in substantial savings for payers.[14]

Lowering Grievance and Administrative Workload: Digital-first, physician-led workflows have the potential to streamline claims, reduce appeals, and expedite resolution of coverage and eligibility matters. Centralizing compliance within clinical teams can lessen communication burdens between parties.

Alignment with CMS Interoperability and Quality Initiatives: Implementing FHIR-based APIs and real-time data exchange supports CMS's interoperability and digital quality reporting objectives. This approach enables seamless, standards-based data sharing, reduces reporting burden for providers and payers, and ensures readiness for evolving CMS requirements.

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[8] 42 CFR § 424.57 (Supplier standards for DMEPOS)
[9] 42 CFR §§ 424.500–424.570 (Medicare enrollment process)
[10] CMS-855S Enrollment Application
[11] CMS, Medicare Program Integrity Manual, Pub. 100-08, Chapter 10: Medicare Enrollment, https://www.cms.gov/regulations-and-guidance/guidance/manuals/internet-only-manuals-ioms/medicare-program-integrity-manual.
[12] 42 U.S.C. § 1395nn (Stark Law)
[13] 42 U.S.C. § 1320a–7b(b) (Anti-Kickback Statute)
[14] AJMC: "ACOs' Focus on Rooting Out Fraud Aligns With CMS Vision Under Oz," April 2025.

C. Technology Integration and API Enhancement

(Responses to PA-1, PA-2, PA-5(a)(b), PR-5(g), PC-8(c)(d), PC-9(a)(c)(d), PC-10(b)(c), PC-14(e)(f), PR-6(b), PR-9, TD-2(a)(b)(c), TD-3, TD-15(a), VB-4)

Blue Button 2.0 API Expansion: We recommend expanding the Blue Button 2.0 API to include:

- Real-time DMEPOS supplier network status and coverage verification
- Product-specific coverage criteria and prior authorization requirements
- Clinical documentation templates and submission tracking
- Patient education materials and training resources

Interoperability Standards: Technology platforms certified to facilitate licensed physicians in the management of DMEPOS would be required to:

- Support interoperability by making available DMEPOS processing data, as facilitated by its participating licensed physician users, via FHIR R4 endpoints. FHIR resources, such as Device, SupplyDelivery, Claim, and DocumentReference and relevant profiles, such as US Core Device Profile must be utilized to standardize data exchange with authorized entities, including bulk access for ACOs.
- Integrate with TEFCA QHINs as a FHIR-enabled participant, using QHIN-facilitated APIs for DMEPOS data exchange while preparing for advanced stages of the FHIR Roadmap.
- Implement IAL2 and AAL2 standards from NIST Special Publication 800-63 regarding trust and authentication.

Requiring use of standards-based authentication protocols (such as OIDC) will enhance cybersecurity and data exchange by enforcing consistent, government-standardized identity proofing and authentication, leaving less room for interpretation and incident. In particular, specific solutions such as Login.gov would further improve cybersecurity by eliminating interpretation and offloading trust entirely to the Login.gov platform.

D. Value-Based Care Integration

(Responses to PC-14(e), VB-1(a), VB-2, VB-3, VB-4, VB-5, VB-9, VB-13, VB-14, VB-15)

Incentivizing Digital Health Adoption in VBC: Direct physician DMEPOS provision, integrated with digital health platforms, aligns incentives for ACOs and other VBC entities by reducing unnecessary utilization, enabling real-time population health analytics, and supporting care coordination. This model empowers clinical teams to act on timely, accurate data, which is essential for shared savings and quality improvement.

Essential Health IT Capabilities and Data Types: The approach leverages FHIR-based APIs and USCDI v3-aligned data exchange to ensure seamless integration with EHRs and payer systems. This supports core VBC functions such as risk stratification, quality measurement, and remote device monitoring, while meeting ONC Health IT Certification Program standards.

Certification, Reporting, and Interoperability: We recommend CMS recognize direct physician DMEPOS provision as meeting enhanced care coordination and reporting requirements for Advanced APMs. Interoperable, API-enabled platforms reduce provider burden, simplify quality reporting, and address common interoperability challenges in VBC programs.

Digital Identity and Nationwide Directories: Adopting IAL2/AAL2 digital identity credentials and participating in FHIR endpoint directories can further strengthen security, streamline patient matching, and improve data access for VBC organizations.

Direct physician DMEPOS provision aligns perfectly with Advanced APM objectives by:

- Reducing unnecessary product utilization through clinical oversight (studies demonstrate up to 30% reduction in product usage)[15]
- Minimizing the frequency of product substitution and non-justified HCPCS code upgrades
- Enhancing care coordination for chronic disease management
- Improving population health outcomes through appropriate product selection

For Advanced APMs, we recommend establishing specific certification criteria that recognize direct physician DMEPOS provision as meeting enhanced care coordination requirements under CEHRT standards.

[15] Author(s). Physician-Managed Distribution of Urological Catheters: A Path to Efficiency. JMIR Preprints. 2025 Jun 6:78670. Preprint. https://preprints.jmir.org/preprint/78670

V. Expected System Improvements and Potential Savings

A. Financial Impact Analysis

(Responses to VB-1(a), VB-14,TD-9(a), VB-3(c) VB-13, VB-15(a))

Fraud Reduction: Based on historical CERT data, the improper payment rate for DMEPOS is 21.4%, totaling approximately $1.9 billion in projected improper payments annually.[16] If transitioning to physician-led DMEPOS services were to reduce improper payments to the rate observed for physician-provided services (approximately 0.5%), modeling indicates the potential reduction would be:

$$\$1.9 \text{ billion} \times ((21.4\% - 0.5\%) / 21.4\%) \approx \$1.82 \text{ billion}$$

For illustrative purposes, if improper payments decreased by 97.5% (from 21.4% to 0.5%), the potential annual savings would be:

$$\$1.9 \text{ billion} \times 0.975 = \$1.85 \text{ billion}$$

This estimate reflects the possible impact of expanded professional oversight in physician practice, which may support a reduction in improper billing opportunities compared to current third-party supplier licensure requirements.

[16] Centers for Medicare & Medicaid Services. "Table A1: 2024 Improper Payment Rates and Projected Improper Payments by Claim Type (Dollars in Billions) (Adjusted for Impact of A/B Rebilling)," in 2024 Medicare Fee-for-Service Supplemental Improper Payment Data, p. 14. https://www.cms.gov/files/document/2024-medicare-fee-service-supplemental-improper-payment-data.pdf

Utilization Optimization: Clinical oversight by physicians supports appropriate utilization and product selection based on clinical need. This approach helps align incentives by addressing patterns such as non-justified HCPCS code upgrades, recommendations for alternative products not consistent with current clinical standards, or the promotion of quantities that exceed patient requirements.

A 2022 pilot study of catheter utilization demonstrated a 30% reduction in median utilization under physician-led models compared to third-party supplier workflows, with no increase in adverse events. While further research is needed to confirm generalizability across DMEPOS categories, these findings suggest a potential for reducing overuse while maintaining care quality.

Savings Calculation (Illustrative):

- Total DMEPOS payments (2024): $9.0 billion (CERT Table A1)
- Proper payments (non-fraudulent): $9.0B – $1.9B (improper) = $7.1 billion
- Potential savings from 23–30% utilization reduction:

$$\$7.1B \times 0.25 \text{ (conservative estimate)} = \$1.78B \text{ annually}$$

Physician oversight may reduce inappropriate code upgrades (e.g., A4351 to A4353) by aligning coding with clinical documentation. CERT data shows incorrect coding contributed to 9.2% of DMEPOS improper payments in 2024, within the overall 21.4% DMEPOS improper payment rate.[17]

Direct provision may also lessen the incidence of alternate product substitutions not aligned with current clinical standards, supporting appropriate item delivery.

Reduced Risk of Adverse Medical Events: More timely DMEPOS delivery and product continuity may reduce avoidable adverse events due to lapses in supply or improper use of a product the patient wasn't trained to use. Evidence from a recent quality improvement project found that optimizing delivery systems decreased treatment delays by 72% and improved clinical outcomes for patients requiring timely access to medical equipment.[18] For value-based entities, even a modest reduction in adverse events such as catheter-associated urinary tract infections (CAUTIs) can yield measurable savings. For example, if a value-based care organization serving 1,000 Medicare beneficiaries with chronic catheter use achieved a 50% reduction in CAUTI rates, this could prevent approximately 10–20 hospitalizations annually, resulting in an estimated $100,000–$200,000 in cost avoidance based on CMS's cost models for avoidable

[17] Centers for Medicare & Medicaid Services. "Table A1: 2024 Improper Payment Rates and Projected Improper Payments by Claim Type (Dollars in Billions) (Adjusted for Impact of A/B Rebilling)," and "Table J2: Improper Payment Rates by Provider Type and Type of Error: DMEPOS," in 2024 Medicare Fee-for-Service Supplemental Improper Payment Data, pp. 14, 62. https://www.cms.gov/files/document/2024-medicare-fee-service-supplemental-improper-payment-data.pdf

[18] 2025 Oxygen Delivery Study: Bayisa, G., Edessa, D., Limenu, K., et al. (2025). Optimising timely and safe oxygen delivery systems for enhanced patient care: a quality improvement project at Wallaga University Comprehensive Specialized Hospital, 2024. BMJ Open Quality. https://bmjopenquality.bmj.com/content/14/1/e003059

hospital-acquired conditions.[19,20] Preventing avoidable (and costly) medical events thus not only saves money but also aligns with the broader value-based care principles championed by CMS.

B. Quality Improvement Outcomes

(Responses to PC-1, PC-4(a)(c), PC-5, PC-8(b)(c)(d), PC-12(a)(b)(c)(d), PC-14(b)(d), TD-2(a)(b)(c), TD-9(a), VB-1(a), VB-3(c), VB-13, VB-14, VB-15(a))

Care Continuity: Physician involvement in DMEPOS provision may enhance care continuity, especially for those with chronic conditions. CMS underscored the importance of physician-led care with the introduction of Chronic Care Management billing codes in 2015, to support care coordination for Medicare beneficiaries.

Accelerated Delivery Times: CMS data indicates that DMEPOS product delivery can take up to 7 days under the traditional third-party supplier model.[21] We have found that physician provision using digital platforms resulted in a substantial reduction in delivery times compared to this industry average. These improvements are primarily due to the elimination of analog processes (such as fax) and the removal of third-party supplier handoffs, streamlining the workflow for both providers and patients.

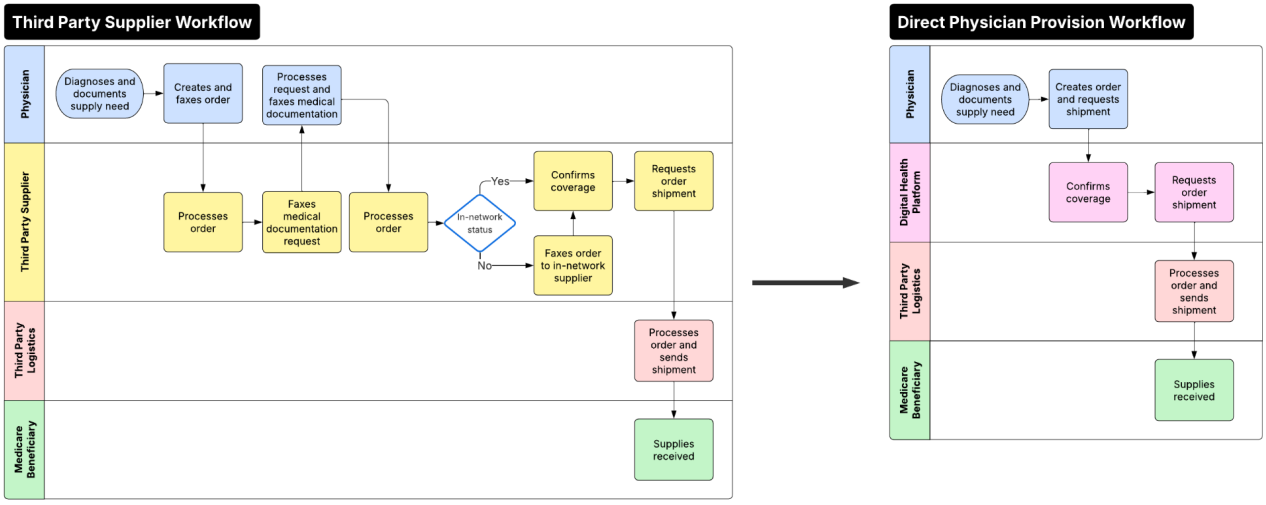

Figure 2: This swimlane diagram contrasts the traditional third-party DMEPOS supplier workflow with the streamlined direct physician provision model.

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[19] CMS, "Fiscal Year (FY) 2022 Medicare Hospital Inpatient Prospective Payment System (IPPS) and Long-Term Care Hospital Prospective Payment System (LTCH PPS) Final Rule," 2021, https://www.cms.gov/newsroom/fact-sheets/fiscal-year-fy-2022-medicare-hospital-inpatient-prospective-payment-system-ipps-and-long-term-care. https://www.cms.gov/newsroom/fact-sheets/fiscal-year-fy-2022-medicare-hospital-inpatient-prospective-payment-system-ipps-and-long-term-care

[20] Economic Evaluation of Quality Improvement Interventions to Prevent CAUTI, BMJ Open Quality 14, no. 1 (2025): e003059; CMS, "FY 2022 IPPS and LTCH PPS Final Rule."

[21] CMS. (2023). Supplemental Report on Intermittent Urinary Catheters, p. 22. https://www.cms.gov/files/document/supplemental-report-intermittent-urinary-catheters.pdf

Patient/Caregiver Voices: The following are individual accounts provided by patients and caregivers, included to reflect qualitative experiences observed in physician-led DMEPOS provision models:

- "I cannot believe the quick turnaround time."
- "I am relieved to receive my order. I had a really bad experience trying to get the script renewed with a third party."
- "Everyone that answers the phone is amazing, goes very smoothly and has been fantastic." "Makes this process so easy."
- "There are many people who don't care for us, but you really care for us and we are so thankful."

C. Elimination of Payer-Provider Workflow Inefficiencies

(Responses to PA-2, PA-5, VB-3(c))

Decreased Reimbursement Ambiguity: Increased centralization of compliance and eligibility information for DMEPOS prescriptions will greatly reduce ambiguity around DMEPOS coverage, resulting in reduced time spent on payer-provider DMEPOS coverage disputes.

VI. Regulatory and Compliance Framework

A. Certification Standards

(Responses to TD-8, TD-9(c)(d), VB-5)

We recommend CMS establish specific certification criteria for physician DMEPOS platforms requiring:

- Automated regulatory compliance monitoring and reporting
- Real-time fraud detection and prevention capabilities
- Clinical need-driven product access assurance
- Patient access API compatibility consistent with CMS Interoperability rules

B. Information Blocking Prevention

(Responses to PC-12(a)(b)(c)(d), PR-12, TD-18(c))

Direct physician provision naturally reduces information blocking by:

- Eliminating third-party supplier gatekeepers who may restrict patient access to product information
- Maintaining transparent clinical decision-making processes
- Ensuring patient access to complete medical records including DMEPOS prescriptions and delivery status

VII. Implementation Recommendations

A. Pilot Program Development

Phase 1 (12-18 months): Establish pilot programs for direct physician DMEPOS provision in high-fraud, waste and abuse categories including urological supplies and diabetes management products. Focus initial implementation on practices already participating in Advanced APMs to leverage existing infrastructure and performance measurement capabilities.

Phase 2 (18-36 months): Expand pilot to additional DMEPOS categories and provider types, incorporating lessons learned and refining compliance frameworks. Develop comprehensive training and certification programs for participating physicians.

B. Long-term Integration

TEFCA Alignment: Integrate physician DMEPOS platforms with TEFCA infrastructure to enable seamless data exchange and care coordination across provider networks.

Quality Measurement: Implement outcome-based reimbursement models that reward quality metrics including patient satisfaction, clinical appropriateness, and cost-effectiveness.

VIII. Conclusion

The current DMEPOS system exhibits inefficiencies that require continued evaluation and improvement. Evidence indicates that some prevailing practices in the sector are associated with patterns of improper billing, increased utilization, product substitutions not aligned with initial clinical prescription, and potential delays in delivery of necessary equipment.

Facilitating direct physician provision of DMEPOS through compliant, integrated technology platforms offers a viable strategy for addressing these challenges. This model has the potential to improve care timeliness and appropriateness, reduce the occurrence of patterns consistent with fraud, waste, or abuse, and support providers in delivering clinically appropriate care.

The proposed reforms are consistent with CMS's broader digital health transformation goals and address specific vulnerabilities that affect the care of Medicare beneficiaries. By reallocating resources from administrative intermediaries toward physicians responsible for clinical care, CMS can support cost optimization as well as improved quality and timeliness in DMEPOS delivery.

We respectfully encourage CMS to consider prioritizing these reforms and welcome the opportunity to collaborate on implementing solutions that enhance service to Medicare beneficiaries and promote system efficiency and integrity.

Conflict-of-Interest Statement: Rx Redefined, Inc. affirms that neither the organization nor its principals have any known financial, professional, or personal conflicts of interest related to the preparation or submission of this response to CMS–0042–NC. This response reflects our independent analysis and recommendations.